



Security: Request for External Security Staff SOP Scope

Site	Department, division or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	All sites and divisions, excluding Next Step Services	Medical, Nursing, Allied Health, Administrative Staff

Links within document

- [Appendix I: External Security Staff Request Form](#)
- [Appendix II: External Security Handover Form](#)

Purpose

This standard operational procedure (SOP) outlines a consistent approach to requesting external security staff both within and after hours, and to ensure timely risk-related assessment, reviews, management strategies and resource allocation.

RPBG recognises while there is 24 hour security on site at Royal Perth Hospital (RPH) and Bentley Health Service (BHS) campuses, there are occasions where external security staff are required to provide attendance for:

- Prolonged time periods for the management of specific risks e.g. on inpatient units
- May be employed to prevent imminent and serious injury to the patient and/or another person(s)/staff and may only be actioned after assertive de-escalation/less restrictive strategies have been trialled or appropriately considered and excluded
- Around the time of a planned outpatient appointment.

Excludes: Next Step Services.

For the purpose of this document patients/consumers across all settings will be referred to as patients.

“Senior clinician/ Senior Registered Nurse (SRN)” definition refers to the following roles for the duration of this SOP:

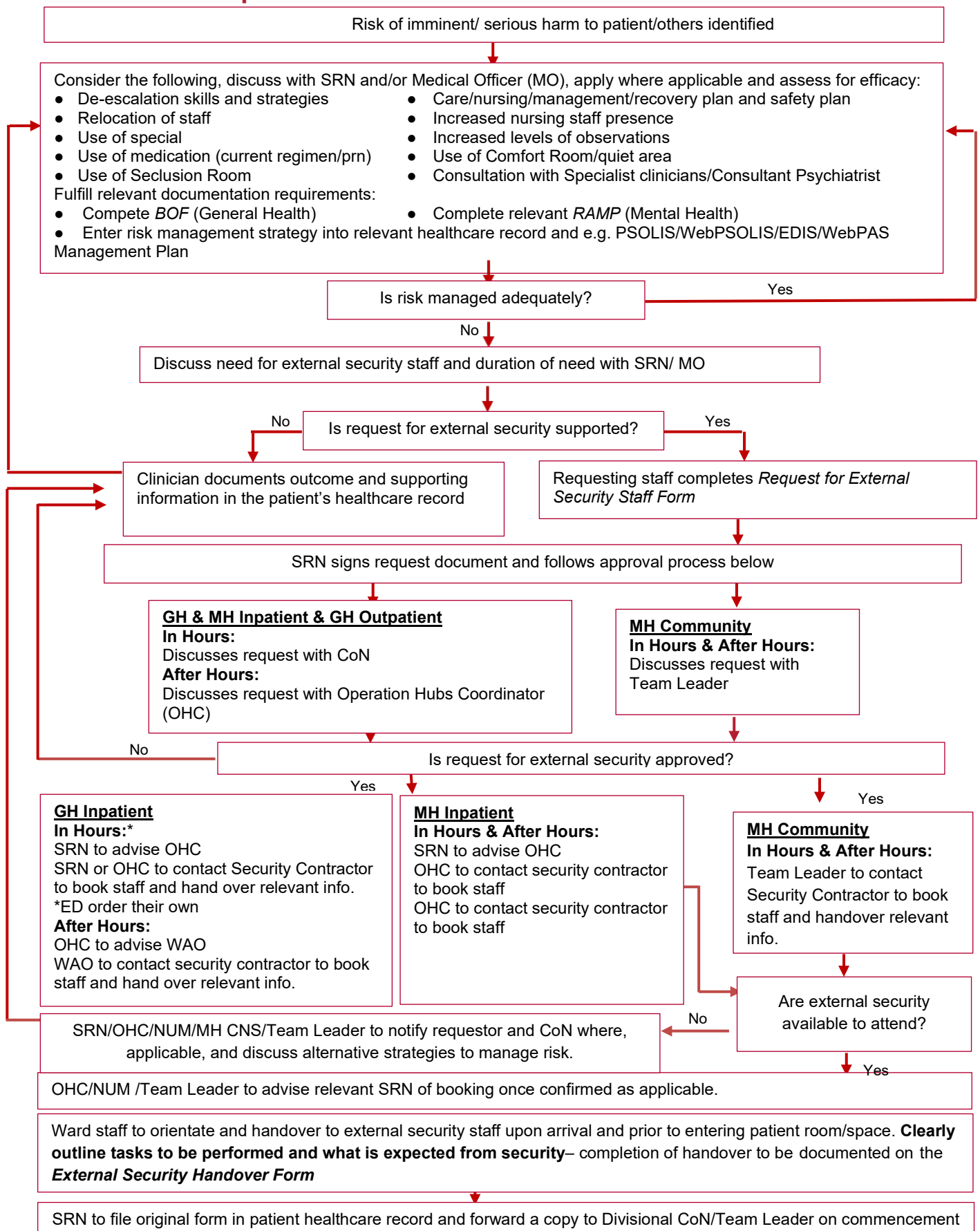
- Nurse Unit Manager (NUM) or Associate NUM
- Midwifery Manager (MM)
- Clinical Nurse Specialist (CNS)
- Clinical Nurse Consultant (CNC)
- Team Leader (community mental health)

Procedure details

- Always provide the least intrusive and restrictive form of care
 - Refer to the [EMHS Reducing Restrictive Practices in Mental Health Services Policy](#) and WA Health - [Use of Restrictive Practices in Non-Authorised Healthcare Settings Policy](#)
- In the event of increasing security-related risk in any clinical area, clinicians are to employ trauma informed care¹, de-escalation skills and alternative patient risk minimisation and care strategies to manage and reduce the risk. Document outcomes in the patient's healthcare record, care plan and *Behavioural Observation Form (BOF)* or *Risk Assessment Management Plan (RAMP)*
 - General Health (GH) areas, refer to Behaviours of Concern - Preventing, Recognising and Responding (excluding Mental Health Division) CPS,
 - Mental Health (MH) areas, refer to [Safety Planning for Mental Health Consumers Policy](#) and [Prevention and Management of Aggressive Behaviours within Mental Health Inpatient Setting SOP](#)
- Ensure escalation for all issues relating to external security to an SRN
- The inpatient team should be informed of the need for additional supervision by external security. A multidisciplinary behavioural management plan should be implemented and documented in the patient's healthcare record
- Refer to Flow Diagrams for [RPH](#) and [BHS](#)
- Consideration for ongoing use of external security staff is to be undertaken prior to the end of each shift (where applicable) to enable continued bookings to be made as per approval process
- Also refer to **Special and Supportive Observations Policy, Recognising and Responding to Acute Deterioration (RRAD) in Mental State, Cognition and Behaviour SOP and RRAD Policy.**

Procedure details cont'd

External security request: Assessment, approval and chain of communication process RPH



Security: Request for External Security Staff SOP

Version: 3

Endorsed by: Communicating for Safety OOS 11/03/2025

Authorised by: Clinical Practice Committee 03/04/2025

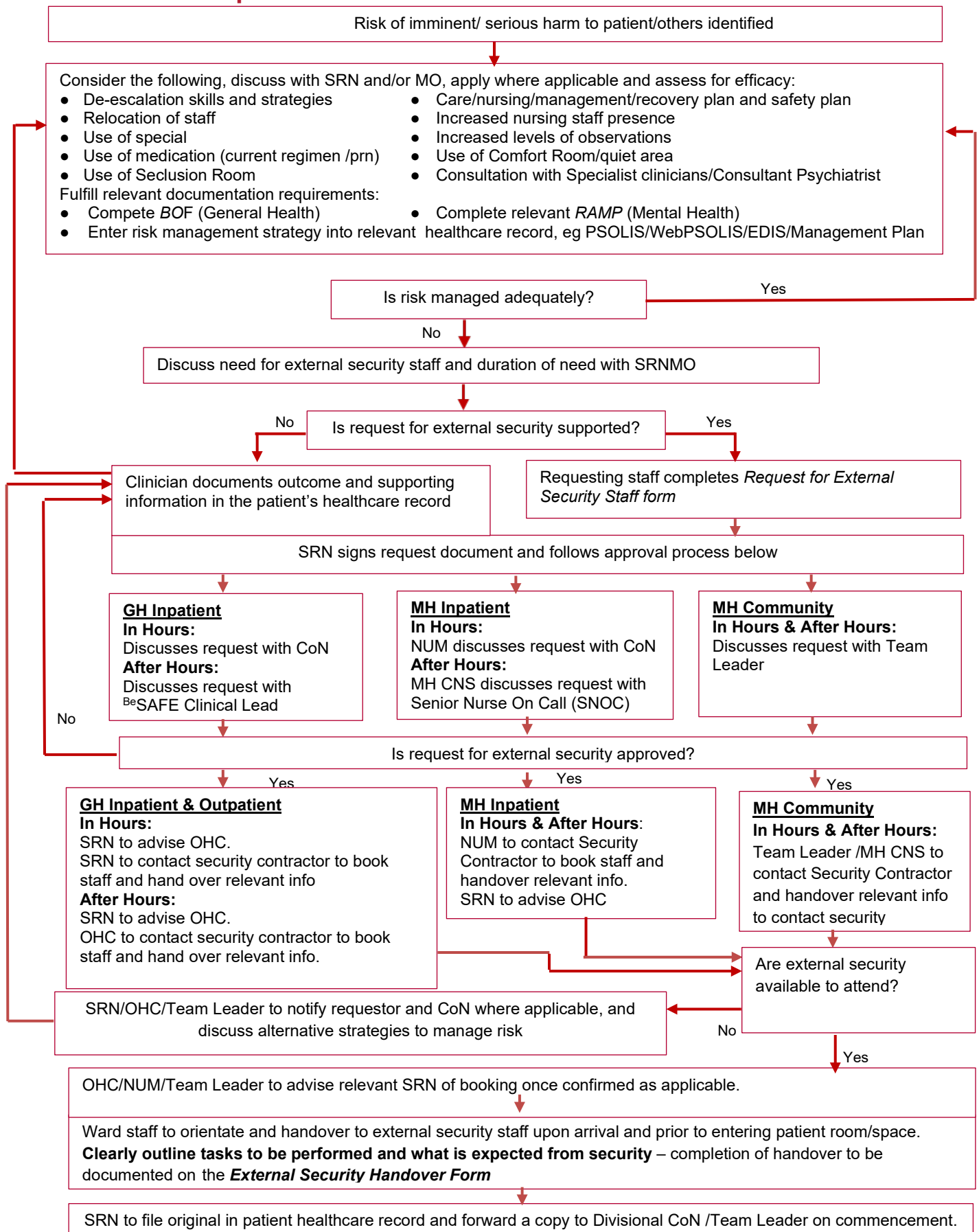
Date Compiled: 09/2020

Executive Sponsor: Director of Nursing, Midwifery and Patient Support Services

Revision Due: 04/2028

Procedure details cont'd

External security request: Assessment, approval and chain of communication process BHS



Security: Request for External Security Staff SOP

Version: 3

Endorsed by: Communicating for Safety OOS 11/03/2025

Authorised by: Clinical Practice Committee 03/04/2025

Date Compiled: 09/2020

Executive Sponsor: Director of Nursing, Midwifery and Patient Support Services

Revision Due: 04/2028

Procedure details cont'd

Continuation of external security

Ongoing assessment of risk, review of strategies and need for external security is to be reviewed on a daily basis as a minimum or every shift where applicable.
 Refer to [RPH](#) or [BHS](#) External Security Request: Assessment, Approval and Chain of Communication Process flowcharts as relevant.
 Document assessment, requests and outcomes in the patient healthcare record and complete the *External Security Staff Request Form*.

Discontinuation of external security

Negotiation is required between SRN and MDT (as required) to identify when risk has been controlled and external security is no longer required.
 Approval process and chain of communication is as request process.
 Document decision/time security discontinued in patient healthcare record.
 Removal of external security
Note: Cost will remain with area utilising external security until that area has communicated external security are no longer required.

Request for external security

Following appropriate reviews or implementation of strategies (as above), the SRN must complete the *Request for External Security Staff form*, noting specified period of time security are required and obtain approval via (refer to [Assessment, Approval and Chain of Communication Flowcharts](#)).

Note: External security approvals out of hours must be reviewed on next working day.

- The decision to use external security with the rationale and associated plan is to be documented in:
 - Patient healthcare record
 - with
 - A risk flag added to iCM for inpatients
 - and
 - An alert added to iCM, Sentry, WebPAS. For MH patients, to Psychiatric Services Online Information System (PSOLIS)/WebPSOLIS
- Original *Request for External Security Staff form* to be filed in patient healthcare record, copy to be forwarded to Divisional CoN/Team Leader.

When booking external security staff, SRN/ Operation Hubs Coordinator (OHC)/Team Leader must ensure:

- Use of [Table 1](#): Security Contractor Booking Contact Details (no preferred provider)
- Information, including any preferred requirements of security staff requested (e.g. gender) and providing as much notice as possible.

Procedure details - Request for external security cont'd

- SRN/OHC/Team Leader must ensure handover is provided to the Security contractor as per the *External Security Handover Form* on a shift to shift basis and prior to any staff changeover and entering patient room/space. Completion of handover is to be documented on the *External Security Handover Form* ([see Appendix II](#)) and filed in the patient healthcare record under "Correspondence"
 - Should the patient require transfer to another health service, handover is to include that external security staff have been utilised and discussion of the potential transfer of security to receiving service
- In the event that external security staff are required for more than one patient, consideration is to be given to allocation of security staff to the ward/service.

Table 1: Security Contractor Booking Contact Details

Company Name:	ERSB - Executive Risk Solutions	VAULT Protective Security Services	NPB Security	Global Protection Security (GPS)
First Contact	Operations Centre: 6165 3333 or 0428 520 846	Operations Manager: 0477 333 046	Requests: 08 6278 5611	Bookings: 0455 906 841
Escalation	Operations Manager: 0458 112 582	Vault Operations: 6279 8237 or 0477 099 099		Business Manager: 0490 021 165
Email	ers247@executiverisksolutions.com.au	operations@vaultsecurity.com.au	work@npbsecurity.com callcentre@npbsecurity.com	
Any issues with providers (e.g. supply/availability), escalate to Area Security Manager				

Compliance monitoring

Compliance will be monitored as part of the Datix CIM process and/or CHoIR/Sentry, the RPBG Performance Monitoring Schedule and via the Compliments or Complaints Management System.

Facilitator

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Authors / acknowledgements

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Previously Guidelines in relation to the request for additional security staff.

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References

1. What is Trauma Informed Care, University at Buffalo, Buffalo Centre for Social Research, The Institute on Trauma and Trauma-Informed Care. Accessed 16/09/2020
<http://socialwork.buffalo.edu/social-research/institutes-centers/institute-on-trauma-and-trauma-informed-care/what-is-trauma-informed-care.html>

Related policies, practice standard, clinical guidelines

RPBG Policy Hub

- [Restraint and Seclusion \(MH\) Policy](#)
- [Safe Use and Management of Patient Restraints CPS RPBG](#)
- [Minimum Requirement for Medical Review of Consumers Admitted to Inpatients Mental Health \(MH\) Wards or CMHS Policy](#)
- [Progressive Risk Assessment SOP \(MH\)](#)
- [Mental Health Inpatient Triage to Discharge SOP](#)
- [General Health Admission, Discharge and Transfer SOP](#)
- [Community Mental Health Services Triage to Discharge SOP](#)
- [Mental Health Consumer/Carer/Visitor Support Post-Critical Incident SOP](#)
- [Behavioural Visual Alerts SOP](#)
- [Clinical Care of People Who May Be Suicidal Policy](#)
- [SAFE and BeSafe SOP](#)
- [Next Step: East Perth After Hours Site Security SOP](#)

Related policies, practice standard, clinical guidelines cont'd

EMHS Policy Hub

- [Prevention and Management of Workplace Aggression and Violence Policy](#)
- [Post Incident Management/Critical Incident Stress Debriefing Policy](#)
- [Security Policy](#)
- [Security Services Manual](#)
- [Emergency and Disaster Management Policy](#)

Department of Health Policy Hub

- [State-wide Standardised Clinical Documentation \(SSCD\) Policy](#)
- [Safety Planning Procedures for Mental Health Consumers](#)
- [Principles and Best Practice for the Care of People Who May Be Suicidal](#)
- [Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#)

Other

- [Mental Health Act \(MHA\) 2014](#)
- [Charter of Mental Health Care Principles \(MHA 2014\)](#)

Related forms

- Behavioural Observation Form (BOF)
- Risk Assessment Management Plan (RAMP)
- External Security Staff Request Form
- External Security Handover Form

Related national standards

ACSQHC NSQHS Standards 2nd Edition (2021):

Standard 1: Clinical Governance

Standard 6: Communicating for Safety

Appendix I: External Security Staff Request Form MR256.1

ROYAL PERTH BENTLEY GROUP		EXTERNAL SECURITY STAFF REQUEST FORM		
PRINT IN BLOCK LETTERS ONLY	WARD/CLINIC		AFFIX LABEL HERE	
	CONSULTANT		SURNAME	MED. REC. NO.
	R.M.O./REG.		FORE NAMES	BIRTH DATE
Clinician Completing Request Form				
Name:				
Designation:		Ward:		
Signature:		Doctor:		
Reason for Request:				
Requested for the following dates and times / shifts:				
Risks:				
What strategies have been utilised?				
For General Health areas, has a Behaviour Observation Form (BOF) been completed? (if YES, attach copy)				
YES / NO Comments (if No)				
For Mental Health areas, has a Risk Assessment and management Plan (RAMP) been completed? (if YES, attach copy)				
YES / NO Comments (if No)				
Is the Risk Management Strategy documented in the EDIS / PSOLIS ward / unit management plan as applicable? (If YES, attach copy)				
YES / NO Comments (if No)				
Has the Medical Officer (MO) / Consultant Psychiatrist or delegate reviewed the patient – including medication regimen? (if YES, specify name and designation)				
YES / NO				
Senior Clinician (e.g. SRN, MO) Approval				
Printed name:				
Designation:				
Signature:		Date:		
Coordinator of Nursing (CoN) / Operation Hubs Coordinator (OHC) / ^{BE}SAFE Clinical Lead / Duty Mental Health (MH) CNS / MH Team Leader				
Printed name:				
Designation:				
Verbal Approval given: YES / NO				
Signature:		Time:	Date:	

Once completed, file in patient's healthcare record and forward copy to Divisional Coordinator of Nursing (CoN) or MH Team Leader on commencement

Appendix II: External Security Handover Form MR256.2

ROYAL PERTH BENTLEY GROUP		EXTERNAL SECURITY HANDOVER FORM			
PRINT IN BLOCK LETTERS ONLY	WARD/CLINIC		AFFIX LABEL HERE		
			SURNAME	MED. REC. NO.	SEX
	CONSULTANT	R.M.O./REG.	FORE NAMES		BIRTH DATE
Ward Clinicians are to ensure handover is completed with external Security Staff including: • Basic patient information • Risks identified • Sexual safety related information • Breaks • Relevant medical conditions • Orientation to Ward / Area • Patient management plan • Triggers and relievers for the patient					
Patient's Name: _____					
Handover Date: ____ / ____ / ____ Time: _____					
Confirmation of handover received					
Security Staff Name: _____					
Signature: _____					
Date: ____ / ____ / ____ Time: _____					
Clinician Name: _____					
Designation: _____					
Signature: _____					
Date: ____ / ____ / ____ Time: _____					
Once completed, file in patient's healthcare record under "Correspondence" and forward copy to Divisional Coordinator of Nursing (CoN) or Mental Health Team Leader on commencement.					

EXTERNAL SECURITY HANDOVER FORM

MR256.2